

So Someone You Love (Might Have) BPD?

How to raise it, what helps in the hardest moments, and how to look after yourself while you do it.



SECTION ONE

Gently bringing up support

When someone you care about experiences intense, fast-shifting emotions or fears of abandonment, suggesting professional help can feel daunting. Bringing up options like Dialectical Behavior Therapy directly can sometimes trigger defensive reactions or fears of rejection if it is not approached thoughtfully. You might consider these strategies when opening a conversation.

- **Choose a calm moment.** You might wait to bring up therapy when both of you are feeling relaxed and grounded, rather than during or immediately after an emotional escalation.
- **Focus on shared goals, not labels.** Rather than framing therapy around a diagnosis or disorder, you could center the conversation on relief: helping them feel less overwhelmed, finding tools to ease emotional pain, or supporting harmony between you.
- **Use “I” statements and reassure security.** You might express your suggestions through your own perspective, for example “I love you and want us to have the best tools to support each other,” to lessen any fear that you are pulling away or judging them.
- **Suggest exploring options together.** You could offer to help look into resources or brief skills-focused programs together, allowing them to feel supported rather than singled out.

SECTION TWO

What to do in moments of distress

Supporting a loved one through intense emotional peaks requires practical self-awareness and steady responses. These are tangible steps you can take to help manage high-stress moments safely.

- **Co-regulate and use sensory grounding.** When stress is very high, logical reasoning often takes a backseat to intense emotional pain. Instead of trying to debate or resolve the issue immediately, you might offer gentle sensory tools, such as bringing them a glass of ice water, offering a cool washcloth, or stepping outside for fresh air, to help soothe their physical nervous system without pressure.
- **Validate emotions while setting boundaries.** You can validate their emotional experience, for example “I can see how hurt and overwhelmed you feel right now,” without agreeing with hurtful behaviors or compromising your own well-being. Setting gentle, clear boundaries, such as taking a temporary fifteen-minute break to cool down, protects both of you and supports long-term emotional stability.

You are not responsible for regulating someone else's emotions.

You can be steady, and you can be kind, and you can still be unable to fix it. That is not a failure on your part. It is the reason structured treatment exists.

If they are in immediate danger

Recurrent suicidal behavior and self-harm are part of the diagnostic picture for BPD. Keep these where you can find them quickly.

- **988 Suicide & Crisis Lifeline.** Call or text **988**. Free and confidential, 24/7 in the US and Canada. You can call about someone else.
- **Crisis Text Line.** Text **HOME** to **741741** to reach a crisis counselor, 24/7.
- **The Trevor Project** (LGBTQ youth). Call **866-488-7386** or text **START** to **678-678**, 24/7.
- If there is immediate risk to life, call **911** and say that the emergency involves a mental health crisis.



The people supporting someone with BPD need supporting too. That is not self-indulgence, it is what makes the support sustainable.

SECTION THREE

Long-term strategies for you

Caring for someone with intense emotional needs works best when you also prioritize your own emotional health and clear communication.

- **Build your own support system.** You might seek out your own therapist, support group, or trusted friends, so that you have a dedicated space to process your own feelings rather than carrying them alone.

- **Practice steady patience.** Progress and skill-building often take time and involve setbacks. Acknowledging small positive changes helps reinforce growth for both of you.
- **Explore flexible resource options.** If full-scale therapy feels like too big a commitment for them right now, you might gently introduce shorter skills-based workbooks or brief community groups as a low-pressure starting point.

Families can learn the same skills.

Family Connections, a program that adapts DBT for relatives, has been trialed with 121 family members, with promising but not yet conclusive results on caregiver burden. The skills in guide two are not only for the person with the diagnosis.

Meet Marley



This project started with one family trying to make sense of a diagnosis.

Marley grew up inside a family where emotions ran high and nobody had a word for why. That question led her to spend a summer buried in clinical research on borderline personality disorder, and to build this series of guides for other families doing the same thing hers did.

Each guide is written from the peer-reviewed literature rather than from anecdote, and every claim is traceable to the sources listed at the back. They are free, and they are meant to be shared with the people who need them.

“Nobody in my family had the language for what was happening. These guides are the thing I wish someone had handed us.”

SOURCES

Works cited

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The Family Connections figure is cited from a trial of 121 family members and is described in the source as promising rather than conclusive. Crisis numbers were verified as current at the time of writing.